pressure occur over the sacrum and coccyx, followed by the trochanter and the calcaneus (heel).

Friction is damaging to the superficial blood vessels directly under the skin. It occurs when two surfaces rub against each other. The skin over the elbows and can be injured due to friction.

Shearing is a separation of the skin from underlying tissues. When a patient is partially sitting up in bed, their skin may stick to the sheet, making them susceptible to shearing in case underlying tissues move downward with the body toward the foot of the bed.

Moisture is also a common pressure ulcer culprit. Sweat, urine, feces, or excessive wound drainage can further exacerbate the damage done by pressure, friction, and shear.

8.6.3 Signs and Symptoms

The early signs of pressure ulcers are

- Unusual changes in skin color or texture
- Swelling
- Tenderness
- Discomfort
- Pus-like draining
- An area of skin that feels cooler or warmer to the touch than other areas
- Local oedema
- Later the area becomes blue purple and mottled
- Due to continued pressure, the circulation is cut-off, the gangrene develops and the affected area is sloughed off..

8.6.4 Stages of Pressure Sores

Stage 1

Intact skin with non-blanch and redness of a localized area usually over a bony prominence.

Stage 2

Partial thickness, loss of dermis presenting as a shallow open ulcer with a red pink wound bed without slough may also present as an intact or open/ruptured serum filled blister. Also presents as a shiny or dry shallow ulcer without slough or bruising. This stage should not be used to describe skin tears, tape burns, perinea dermatitis, maceration or excoriation

Stage 3

Full thickness, tissue loss, subcutaneous fat may be visible but bone, tendon or muscle are not exposed.

Stage 4

Full thickness tissue loss with exposed bone, tendon or muscle. Slough or eschar may be present on some parts of the wound bed.

Unstageable

Full thickness tissue loss in which actual depth of the ulcer is completely obscured by slough (yellow, tan, gray, green or brown) and/or eschar (tan, brown or black) in the wound bed. Until enough slough and/or eschar is removed to expose the base of the wound, the true depth, and therefore stage, cannot be determined.

Suspected Deep Tissue Injury

A purple or maroon localized area of discoloured intact skin or blood-filled

Nursing-Gen_Chapter 08.indd 157 07/05/18 12:56 PM